CALIFORNIA SEX OFFENDER MANAGEMENT BOARD
9838 Old Placerville Road
Sacramento, CA 95827

COMPLAINT FORM

Submission of this form means that you are filing a formal complaint with the California Sex Offender Management Board (CASOMB) about a CASOMB certified provider or provider agency. Please complete this form as instructed in its entirety. Incomplete or insufficiently detailed forms will be returned without review. Upon receipt of the completed form, CASOMB will review the complaint according to the most recent version of the Sex Offender Treatment Provider Certification Requirements. CASOMB can only respond to violations of the Sex Offender Treatment Provider and/or Provider Agency Requirements, posted online at www.CASOMB.org under the Certification tab.

COMPLAINANT'S NAME:		*PHONE:*
Brendan Ngehsi Newanforbi		(209) 801-4442
ADDRESS:	*CITY:*	*ZIP:*
1455 Cat Tail Drive	Stockton, CA	95204
E-MAIL:		
bnn.cdl@gmail.com		
SERVICE PROVIDER/Provider Agency (A CASOMB listed provider must be named):		*PROVIDER PHONE:*
HOPE Psychotherapy, Inc. (d/b/a " HOPE Program ")		(510) 764-2428
ADDRESS:	*CITY:*	*ZIP:*
5403 N. Pershing Avenue, Suite 200	Stockton, CA	95207

I. NATURE OF COMPLAINT:

Please provide a description of the situation or circumstances related to the violation of the requirements. Please refer to the Sex Offender Treatment Provider and/or Program Certification Requirements for details. Continue on a separate sheet if needed and attach any supporting documentation or verification.

This complaint is filed against HOPE Psychotherapy, Inc. (" HOPE Program "), a CASOMB-certified sex offender treatment provider agency, and its Clinical Director, Katherine A. Palacios, LMFT (License No. 44555). HOPE contracts with the California Department of Corrections and Rehabilitation (" CDCR ") to provide mandatory sex offender treatment services under California Penal Code § 3008(d). The HOPE Program operates at 5403 N. Pershing Avenue, Suite 200, Stockton, CA 95207.

This complaint arises from multiple categories of violations of the CASOMB Provider Agency Certification Requirements (January 2019), including:
(a) Application of a blanket retention policy in lieu of individualized clinical assessments, in violation of Requirements 7 and 9;
(b) Failure to comply with ethical standards, including retaliation against a client for protected expression, in violation of Requirement 2, item 4;
(c) Operation of an arbitrary treatment structure with no published criteria for phase progression, completion, or appeal, in violation of Requirements 7 and 9;

II. STANDARD(S) VIOLATED:

The specific CASOMB Requirement(s) must be cited and how they were allegedly violated by the provider. Please refer to the Sex Offender Treatment Provider and/or Program Certification Requirements for details. Continue on a separate sheet if needed.

The following specific CASOMB Provider Agency Certification Requirements (January 2019) have been violated by HOPE Program and its Clinical Director, Katherine A. Palacios:

REQUIREMENT 2: Basic Operating Policies and Practices

Item 4 — Compliance with Ethical Standards (ATSA Code of Ethics): Ms. Palacios made false, defamatory, and retaliatory statements about me at a CTM meeting in the presence of parole officials, in direct response to my raising legitimate complaints about program conditions. She later confirmed the retaliatory nature by praising me when I ceased exercising those rights. Retaliating against a client for protected expression violates the ethical standards required of treatment

III. STEPS TAKEN TO RESOLVE THE COMPLAINT:

CASOMB believes that most complaints can be resolved by addressing the issue directly with the source of the complaint. What steps have you taken to resolve this complaint at the Containment Team Level?

I have taken the following steps to resolve these issues at the Containment Team level and beyond:

1. I raised my concerns about program conditions directly during HOPE group sessions in 2024 and 2025, including my prolonged retention, the loss of homework assignments by program staff, and the disruption caused by clinician turnover and group resets. Rather than address these concerns, Ms. Palacios retaliated by making defamatory statements about me at the August 2025 CTM meeting.
2. Following the retaliation, I curtailed my speech during group sessions and abandoned further

IV. DESIRED OUTCOME:

Please describe what you would like to occur in order to resolve the situation. Continue on a separate sheet if needed.

I respectfully request that CASOMB take the following actions:

1. Investigate HOPE ' s blanket retention policy and determine whether it violates the CASOMB Treatment Completion Guidelines, the Maintenance and Aftercare guidelines, and the Provider Agency Certification Requirements for individualized treatment planning.
2. Conduct an on-site compliance review of the HOPE Program Stockton facility, including an inspection of facility conditions (restrooms, stairwells, common areas) and a review of client files for compliance with individualized treatment planning and documentation requirements.

Thank you for addressing your concerns to the Board. You will be notified in writing if further information is required e.g., a HIPAA release for confidential treatment information. You will receive written notification of the decision. Names will only be disclosed to the extent necessary to resolve the complaint.

Name	Signature	Date
Brendan Ngehsi Newanforbi		02/21/2026

CASOMB COMPLAINT FORM — CONTINUATION SHEET

Brendan Ngehsi Newanforbi v. HOPE Psychotherapy, Inc.

Date: February 21, 2026

I. NATURE OF COMPLAINT (Continued)

[Continued from Complaint Form, Section I. Items (a) through (c) appear on the form. Items (d) through (f) and the full narrative continue below.]

(d) Failure to maintain accurate records of client progress, including routine loss of homework assignments, in violation of Requirement 10;

(e) Maintenance of a treatment facility in unsanitary and unsafe conditions, in violation of Requirement 2 (basic operating policies and appropriate business conditions); and

(f) Financial conflict of interest undermining clinical objectivity, with revenue structures that incentivize indefinite client retention rather than treatment completion.

A. Blanket Retention Policy

HOPE maintains an official policy of refusing to certify "completion" for any parolee prior to discharge from parole, regardless of clinical progress, compliance, risk reduction, or time in treatment. This policy was documented under oath by HOPE's own Executive Director of Clinical Training in the case of *In re Frederick Dwyer*, No. SC-080355A (San Mateo Superior Court, March 20, 2023), where the court found this practice "vitiates the Legislature's directive" under Penal Code § 3008(d)(2). Despite this judicial condemnation, Ms. Palacios has continued applying the identical policy to my treatment through November 2025 and beyond.

Effective April 17, 2025, Title 15 CCR § 3574(f)(1) explicitly provides that "Treatment completion and parole discharge are legally separate decisions." A treatment provider that refuses to certify completion until discharge from parole collapses these separate decisions into one, in direct violation of both this regulation and CASOMB's own Treatment Completion Guidelines (2018), which require individualized assessment of risk level, criminogenic needs, and individual factors.

Ms. Palacios represented to CDCR—through treatment status reports, CTM inputs, and the November 2025 Discharge Review Report—that I have "not completed" sex offender treatment. This characterization was not the product of an individualized clinical assessment. It was the automatic output of HOPE's blanket retention policy, which makes completion impossible for every participant prior to parole discharge without exception.

B. Retaliatory and Defamatory Statements at CTM Meeting

In or about August 2025, during a Case Conference/Containment Team Meeting ("CTM") held at a CDCR parole office in Stockton, California, Ms. Palacios made false, defamatory, and retaliatory statements about me in the presence of multiple parole agents. The following individuals were present: Agent Chow (Parole Agent, CDCR); Agent G. Noguchi #7284 (Supervising Parole Agent, CDCR); Agent A. Derrick #6910 (my assigned Parole Agent, CDCR); the then-current supervising parole agent (now retired); and Ms. Palacios.

After initially acknowledging my respectful conduct during the meeting, Ms. Palacios stated, in substance, that I was "disruptive during HOPE group sessions," a "ringleader," that my behavior

was “abhorrent,” and that I was “often disrespectful to [her] staff.” She characterized my respectful demeanor at the CTM as an “acting job.”

These statements were false. My conduct in group sessions consisted of raising legitimate concerns about program conditions, including: my prolonged retention in the program despite completion of all assigned coursework; the repeated loss of my completed homework by program staff; and the disruption of my progress caused by repeated clinician turnover and resulting group resets. Other participants who independently shared similar concerns were falsely attributed to my “influence.” Ms. Palacios’s own words confirm the retaliatory nature: “because when you start complaining then the other guys chime in”—i.e., others voluntarily agreed with my protected expression.

Following this incident, I curtailed my speech during HOPE group sessions, abandoned written complaints, and experienced significant anxiety and emotional distress. Ms. Palacios subsequently praised me for the change—confirming that her prior statements were designed to suppress advocacy and punish protected expression.

C. Arbitrary Treatment Structure With No Published Criteria

Under Ms. Palacios’s direction, HOPE’s treatment structure requires mandatory phase repetitions with no basis in statute, regulation, or CASOMB requirements. Participants must complete the “Core” phase twice, the “Maintenance” phase twice, and then a “titration” phase before being considered for program completion. This framework appears nowhere in Penal Code § 3008(d) or Title 15 CCR § 3574, and results in participants remaining in treatment for 5–7+ years with no meaningful prospect of completion.

Ms. Palacios has never provided me with: (a) written criteria for program completion; (b) a written explanation of what conduct or progress is required to advance through phases; (c) an appeal process for titration or phase-advancement decisions; or (d) any mechanism to challenge lost homework or forced phase restarts.

D. Failure to Maintain Accurate Records

Ms. Palacios has admitted that homework assignments are “routinely lost” within the HOPE Program. Despite this admission, she has failed to implement any safeguards against record loss. Participants—including me—are forced to restart treatment phases due to staff turnover or job-related group changes, forfeiting progress previously earned. This constitutes a failure to maintain accurate and complete documentation of client progress.

E. Unsanitary and Unsafe Facility Conditions

The HOPE Program facility at 5403 N. Pershing Avenue, Suite 200, Stockton, CA 95207 is maintained in conditions that are inconsistent with a professional treatment environment. Photographic evidence (attached as Exhibits) documents the following conditions at the facility:

> **Restroom Conditions:** Severely unsanitary restrooms including clogged and overflowing toilets filled with waste and debris; dirty and stained tile floors with visible grime; trash and food packaging dumped in sinks; unclean countertops with spills and biological matter. (See Exhibits: IMG_4599 through IMG_4612.)
>
> **Stairwell and Building Access:** Dark, poorly lit, or completely unlit interior stairwells that present a safety hazard, particularly for clients attending evening treatment sessions.

The exterior entrance areas are neglected and contain litter. (See Exhibits: IMG_4597, IMG_4598, IMG_4613, IMG_4614.)

These conditions exist at a facility that bills $100.00 per 90-minute group psychotherapy session, $200.00 per 60-minute individual session, $500.00 per polygraph examination, and $1,800.00 per psychosexual evaluation. The contrast between the fees charged and the conditions maintained raises serious concerns about the agency's commitment to providing professional treatment services.

F. Financial Conflict of Interest

HOPE operates under a fee-for-service billing model. Per the Alameda County Probation Department Procurement Contract No. 26374 (First Amendment approved January 9, 2024), HOPE bills: $100.00 per group psychotherapy session (90 minutes); $100.00 per Collaborative Model Team meeting (per client staffed, 90 minutes); $200.00 per individual psychotherapy session (60 minutes); $500.00 per Polygraph Examination; $1,800.00 per psychosexual evaluation; and $125.00 per quarterly treatment plan.

A single parolee attending two group sessions per week generates approximately $10,600 annually in baseline group and CTM fees alone. Over a 10-year parole term, this represents approximately $106,000 in baseline revenue from continued enrollment. HOPE's contracts provide no documented compensation, bonus, or incentive for successful treatment completion. Revenue terminates upon discharge. Ms. Palacios, as Clinical Director, controls the "completion" determination that CDCR uses as a gatekeeper for discharge eligibility, while possessing a direct, ongoing financial interest in ensuring that determination is never favorable.

II. STANDARD(S) VIOLATED (Continued)

[Continued from Complaint Form, Section II. The form contains the beginning of Requirement 2, Item 4. The full text of all violated requirements continues below.]

REQUIREMENT 2: Basic Operating Policies and Practices

Item 4 — Compliance with Ethical Standards (ATSA Code of Ethics): Ms. Palacios made false, defamatory, and retaliatory statements about me at a CTM meeting in the presence of parole officials, in direct response to my raising legitimate complaints about program conditions. She later confirmed the retaliatory nature by praising me when I ceased exercising those rights. Retaliating against a client for protected expression violates the ethical standards required of treatment providers, including the ATSA Code of Ethics.

Item 5 — Treatment Provider Boundaries: Using the therapist-client relationship to punish protected speech and suppress advocacy constitutes an abuse of professional authority and a violation of treatment provider boundaries with offender clients.

Item 10 — Monitoring Compliance with CASOMB and SARATSO Requirements: HOPE has failed to monitor its own compliance with CASOMB requirements, including the Treatment Completion Guidelines, the Maintenance and Aftercare guidelines, and the requirement for individualized treatment planning.

General — Appropriate Business Conditions: The CASOMB Certification Requirements state that agencies must have "appropriate business licenses for their locations." The HOPE facility is maintained in severely unsanitary and unsafe conditions (unsanitary restrooms, dark unlit stairwells, neglected premises) that are inconsistent with a professional treatment environment and raise questions about compliance with basic health and safety standards. Photographic evidence is attached.

REQUIREMENT 5: Informed Consent, Waiver of Confidentiality, and Treatment Contract

Section A, Item 12 — Client Fees: HOPE operates under a fee-for-service billing model that creates a structural financial conflict of interest. Revenue terminates upon discharge, providing a direct financial incentive to retain clients indefinitely rather than certify treatment completion. This conflict was not disclosed to me.

Section D — Treatment Contract (Items 1 and 5): The treatment contract must "describe the type, frequency, and requirements of the treatment, and outline how the duration of treatment will be determined" (item 1) and "describe the provider agency's grievance process to address and resolve client complaints" (item 5). HOPE failed to provide written criteria for treatment duration, phase progression, or completion. HOPE also failed to provide a meaningful grievance process—raising complaints about program conditions resulted in retaliation rather than resolution.

REQUIREMENT 7: Written Treatment Plan

CASOMB requires individualized written treatment plans. HOPE applied a blanket retention policy—making completion impossible for all participants prior to parole discharge—rather than maintaining individualized treatment plans with achievable goals and measurable completion criteria. My treatment status characterization was the automatic output of this blanket policy, not the product of individualized clinical assessment. This violates both the

requirement for individualized written treatment plans and the CASOMB Treatment Completion Guidelines (2018), which require that "the determination of treatment completion" consider risk level (Guiding Principle 11), individual factors (Principle 12), predicted trajectory (Principle 13), and containment team judgment (Principle 14).

REQUIREMENT 9: Treatment (Including Treatment Completion)

Treatment Completion Guidelines (2018): HOPE's blanket retention policy violates the Treatment Completion Guidelines, which require individualized assessment based on risk level, criminogenic needs, and the RNR (Risk, Need, Responsivity) principles. The guidelines explicitly state that treatment completion is a "protective factor" associated with reduced risk (Principle 16), and that the determination should be a "YES-or-NO conclusion about whether treatment has been completed" (Principle 20).

Maintenance and Aftercare Guidelines (September 2024): The Maintenance and Aftercare guidelines require that "discharge planning starts at intake" and that "the final determination regarding whether an individual has 'completed' treatment should never come as a surprise to the client." HOPE's blanket retention policy, combined with its unpublished criteria, ensures that completion is both a surprise and an impossibility.

REQUIREMENT 10: Other Documentation

Ms. Palacios has admitted that homework assignments are "routinely lost" within the HOPE Program. Despite this admission, no safeguards have been implemented. Participants are forced to restart treatment phases due to staff turnover or lost work, forfeiting progress previously earned. This constitutes a failure to maintain accurate and complete documentation of client progress as required by CASOMB certification standards.

REQUIREMENT 11: Complaints Against Treatment Providers and/or Provider Agencies

Requirement 11 mandates that the provider agency maintain an internal grievance procedure and notify offender clients of the CASOMB complaint process. Ms. Palacios retaliated against me for raising legitimate complaints about program conditions during group sessions, thereby chilling the exercise of the grievance and complaint processes that CASOMB requires agencies to maintain. Per the CASOMB Complaint Procedure (2025), item 19: "Respondent providers or provider agencies may not discriminate or retaliate against complainants who filed a complaint in good faith."

III. STEPS TAKEN TO RESOLVE THE COMPLAINT (Continued)

[Continued from Complaint Form, Section III. Items 1-2 begin on the form. The full list continues below.]

1. I raised my concerns about program conditions directly during HOPE group sessions in 2024 and 2025, including my prolonged retention, the loss of homework assignments by program staff, and the disruption caused by clinician turnover and group resets. Rather than address these concerns, Ms. Palacios retaliated by making defamatory statements about me at the August 2025 CTM meeting.

2. Following the retaliation, I curtailed my speech during group sessions and abandoned further written complaints to HOPE, as the retaliation made clear that internal complaint mechanisms would result in further adverse consequences.

3. I filed a complaint with the California Board of Behavioral Sciences (BBS) against Ms. Palacios (LMFT License No. 44555) regarding her professional misconduct.

4. I filed a federal civil rights action: *Newanforbi v. Derrick, et al.*, Case No. 2:26-cv-00004-DJC-JDP (E.D. Cal.), which includes claims arising from the same course of conduct.

5. I am now filing this CASOMB complaint as the appropriate forum for addressing violations of the CASOMB Provider Agency Certification Requirements.

IV. DESIRED OUTCOME (Continued)

[Continued from Complaint Form, Section IV. Items 1-2 begin on the form. The full list continues below.]

1. Investigate HOPE's blanket retention policy and determine whether it violates the CASOMB Treatment Completion Guidelines, the Maintenance and Aftercare guidelines, and the Provider Agency Certification Requirements for individualized treatment planning.

2. Conduct an on-site compliance review of the HOPE Program Stockton facility, including an inspection of facility conditions (restrooms, stairwells, common areas) and a review of client files for compliance with individualized treatment planning and documentation requirements.

3. Require HOPE to provide all current participants with written, achievable criteria for phase progression, treatment completion, and any appeal or challenge mechanisms, in compliance with the Treatment Contract requirements (Requirement 5, Section D) and Written Treatment Plan requirements (Requirement 7).

4. Require HOPE to implement safeguards against the routine loss of client homework and progress documentation, and to ensure that participants are not forced to restart treatment phases due to the agency's administrative failures.

5. Require HOPE to bring the Stockton facility into compliance with professional standards for a treatment environment, including sanitary restrooms and adequately lit stairwells and common areas.

6. Take appropriate disciplinary action against HOPE and/or Ms. Palacios for the violations identified herein, which may include a letter of admonition, probation, suspension, or decertification as warranted by the findings.

7. Refer any findings of criminal and/or unethical behavior to the appropriate licensing board (California Board of Behavioral Sciences) and/or law enforcement agency.

V. SUPPORTING DOCUMENTATION AND EXHIBITS

The following supporting documentation and evidence is attached to or accompanies this complaint:

Exhibit A: Photographic evidence of unsanitary restroom conditions at the HOPE Stockton facility (IMG_4599 through IMG_4612)

Exhibit B: Photographic evidence of dark, unlit stairwells and neglected building exterior (IMG_4597, IMG_4598, IMG_4613, IMG_4614)

Exhibit C: HOPE Program Fee Model documentation

Exhibit D: Discharge Review Report (November 2025)

Exhibit E: BBS Complaint against Katherine A. Palacios, LMFT License No. 44555

Thank you for addressing these concerns to the Board. I am available to provide any additional information or documentation required to investigate this complaint.

Respectfully submitted,

/s/ Brendan Ngehsi Newanforbi
February 21, 2026

EXHIBIT A

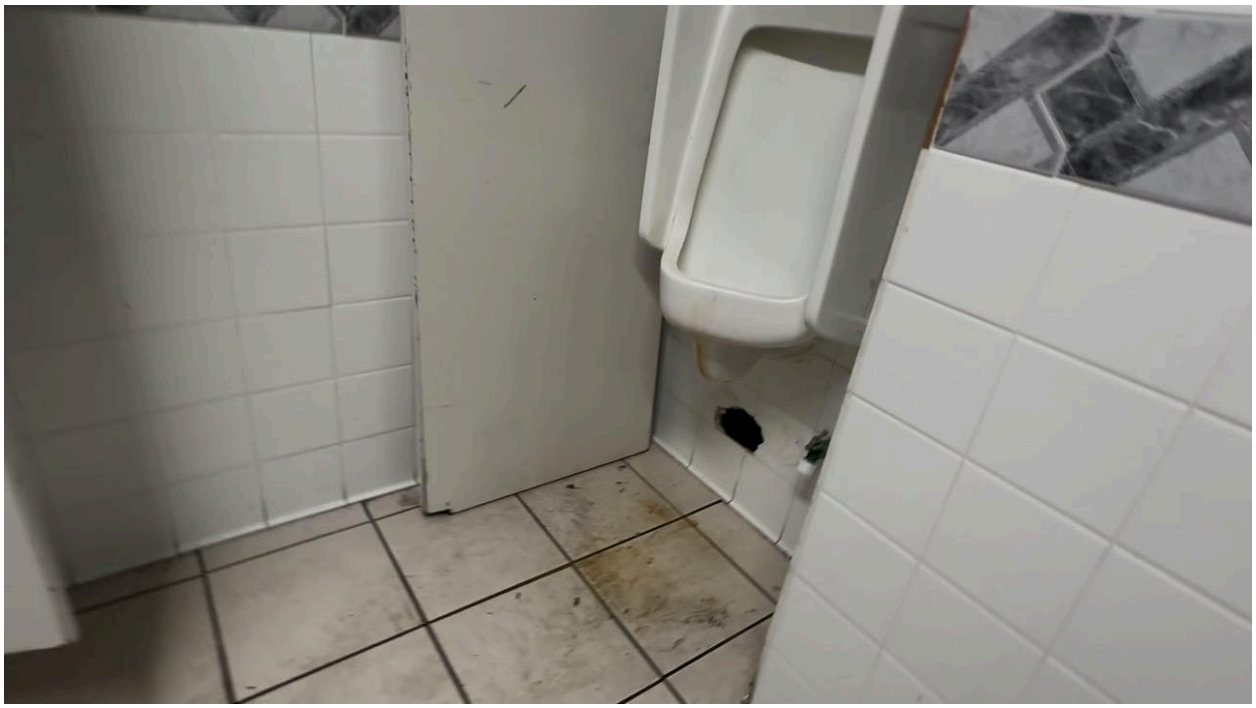

EXHIBIT B

EXHIBIT C

AGENDA #_____, January 9, 2024

ALAMEDA COUNTY PROBATION DEPARTMENT

P.O. Box 2059
1111 Jackson Street
Oakland, CA 94604-2059

MARCUS DAWAL
Chief Probation Officer

December 21, 2023

Honorable Board of Supervisors
County of Alameda
1221 Oak Street, Suite 536
Oakland, California 94612-4305

SUBJECT: APPROVE A FIRST AMENDMENT WITH HOPE PSYCHOTHERAPY, INC. FOR SEX OFFENDER THERAPY SERVICES; PROCUREMENT CONTRACT NO. 26374; AMOUNT: $100,000

Dear Board Members:

RECOMMENDATION:

Approve a First Amendment to Standard Services Agreement (Procurement Contract No. 26374) with HOPE Psychotherapy Inc. (Principal: Shannon Smith; Location: Hayward), to provide youth sex offender treatment services, extending the current term of 7/1/23 – 6/30/24 by one year until 6/30/25, and increasing the contract amount from $25,000 to $100,000 ($75,000 increase).

DISCUSSION/SUMMARY:

The Alameda County Probation Department (ACPD) contracts with HOPE Psychotherapy, Inc. (HOPE) for youth sex offender treatment services. HOPE works with youth and young adult clients in individual and group settings. The program specializes in forensic and cognitive behavioral therapy, as well as general psychotherapy for individuals, couples, and families coping with emotional, behavioral, and psychological challenges. In the past year, the program has seen an increase of young adults being ordered to sex offender treatment who are older than 18.

The increase in the contract amount is being requested due to the increased amount of court orders for treatment services for youth and young adults. The treatment time for program participants is at least one year. During this amendment term, ACPD will work with General Services Agency to complete a formal bid to secure a long-term contract.

SELECTION CRITERIA/PROCESS:

ACPD has determined that the County does not currently have the resources to provide sex offender treatment services.

ACPD issued an Informal Request for Quote (IRFQ) in March 2022. Two responses to the IRFQ were received and evaluated. HOPE, a local vendor, was the lowest qualified vendor who met all the requirements of the IRFQ. ACPD is satisfied with the services provided by HOPE.

To provide sex offender treatment, agencies need to be certified by the California Sex Offender Management Board (CASOMB). In Alameda County, there are only five agencies certified by CASOMB. ACPD did not receive any bid responses from certified Small Local Emerging Businesses. Due to the nature of this unique service and limited certified agencies in Alameda County, subcontracting opportunities are not available. As a result, the SLEB requirements have been waived by the Office of Acquisition Policy (SLEB Waiver No. 9187, expires 6/30/2025).

FINANCING:

Funding for this contract amendment is included in ACPD Fiscal Year 2023-24 Approved Budget. No additional funding is required and there will be no increase in net County costs.

VISION 2026 GOAL:

Youth sex offender treatment services meet the 10X goal pathway of a **Crime Free County** in support of our shared vision of a **Safe & Livable Communities.**

Respectfully submitted,

Marcus Dawal
Chief Probation Officer

MD:yw

Procurement Contract No. 26374

COUNTY OF ALAMEDA
STANDARD SERVICES AGREEMENT

This Agreement, dated as of July 1, 2023, is by and between the County of Alameda, hereinafter referred to as the "County", and Hope Program, A Licensed Clinical Social Worker Professional Corporation dba Hope Psychotherapy, Inc., hereinafter referred to as the "Contractor".

WITNESSETH

Whereas, County desires to obtain Youth Sex Offender Treatment services which are more fully described in Exhibit A hereto ("Sex Offender Treatment"); and

Whereas, Contractor is professionally qualified to provide such services and is willing to provide same to County; and

Now, therefore it is agreed that County does hereby retain Contractor to provide Sex Offender Treatment, and Contractor accepts such engagement, on the General Terms and Conditions hereinafter specified in this Agreement, the Additional Provisions attached hereto, and the following described exhibits, all of which are incorporated into this Agreement by this reference:

Exhibit A	Definition of Services
Exhibit B	Payment Terms
Exhibit C	Insurance Requirements
Exhibit D	Debarment and Suspension Certification
Exhibit E	Prison Rape Elimination Act Certification

The term of this Agreement shall be from July 1, 2023 through June 30, 2025.

The compensation payable to Contractor hereunder shall not exceed *One Hundred Thousand dollars* (*$100,000*) for the term of this Agreement.

IN WITNESS WHEREOF, the parties hereto have executed this Agreement as of the day and year first above written.

COUNTY OF ALAMEDA

By: _____
Signature

Name: _____
(Printed)

Title: President of the Board of Supervisors

Date: _____

HOPE PROGRAM, A LICENSED CLINICAL SOCIAL WORKER PROFESSIONAL CORPORATION dba HOPE PSYCHOTHERAPY, INC.

By: DocuSigned by: Shannon Smith 3850FE5A4[illegible]
Signature

Name: Shannon Smith
(Printed)

Title: CEO

Date: 12/26/2023

Approved as to Form:
Donna R. Ziegler, County Counsel

By: DocuSigned by: K. Joon Oh EFBCE3E[illegible] 12/20/2023
K. Joon Oh, Deputy County Counsel

By signing above, the signatory warrants and represents that he/she executed this Agreement in his/her authorized capacity and that by his/her signature on this Agreement, he/she or the entity upon behalf of which he/she acted, executed this Agreement

EXHIBIT B

PAYMENT TERMS

1. County will use its reasonable efforts to make payment to Contractor upon successful completion and acceptance of the following services listed within thirty (30) days upon receipt and approval of invoice.

Description of Services	Cost
Psychosexual Evaluation	$1,800.00
Individual Psychotherapy – 60 minutes	$200.00
Group Psychotherapy – 90 minutes	$100.00
Family Psychotherapy – 60 minutes	$185.00
Collaborative Model Team meeting (CMT) – 90 minutes	$100.00
Chaperone Training	$200.00
Visual Reaction Time Measure of Sexual Interest and Report	$500.00
Quarterly Treatment Plans	$125.00

No shows, whether excused or unexcused, cannot be billed

2. County will use its best efforts to make payment to Contractor within thirty (30) days upon receipt, review, and approval of invoice with acceptable back-up documentation, Contractor shall provide additional backup documentation and other information and records as requested by the County.

3. Invoices shall be submitted along with the required statement of completed services.by the 10th of each month (or the next business day with the 10th is on a weekend or holiday) upon completion of work.

   a. Invoices shall be submitted via ACPD's Finance Portal at https://fiscal.acgovpb.org.

   b. Invoices will be reviewed for accuracy and approval by the County, Probation Department.

   c. Services have been provided and completed during the term of the contract.

   d. The service type provided, rates are consistent with, and comply with, the terms of the contract.

   e. The amounts on the invoice are mathematically correct.

4. Total payment under the terms of this Agreement will not exceed the total amount of $100,000. This cost includes all taxes and all other charges.

EXHIBIT D

STATE OF CALIFORNIA
DISCHARGE REVIEW REPORT
CDCR 1502-DR (Rev 01/22)

DEPARTMENT OF CORRECTIONS AND REHABILITATION
DIVISION OF ADULT PAROLE OPERATIONS

DISCHARGE REVIEW REPORT

SECTION I PAROLEE PROFILE Case Type: **PC3000(b)(5)(B)**

CDC Number:	Print Name (Print Last, First, MI):	Region:	Parole Unit:
BF3571	**NEWANFORBI, BRENDAN, NGEHSI**	**N**	**STOCKTON 2 GPS**

CSRA Score:	Supervision Category:	Discharge Review Date:	Controlling Discharge Date:
2	**SA**	**12-01-2025**	**12-02-2030**

Commitment Offenses:
PC289(a)(1)(A)[01] Penetration w/Force/Violence/Fear of Immediate Bodily Injury, PC220(a)(1)[01] Assault with Intent to Commit Specific Sex Offense

Imminent Discharge: [X] DNA Sample Obtained: [X] YES [ ] NO Non-Controlling Offenses: [X] YES [ ] NO

Commitment Offense Subject To PC 667.5(C): [X] YES [ ] NO Commitment Offense Subject To PC1192.7 9 (C) or 1192.8: [X] YES [ ] NO

PC 290 Registrant: [X] YES* [ ] NO * If Yes, Sex Offender Static Risk Assessment: Score: 4

Other Registration Requirements: [ ] PC 457.1 [ ] PC 186.30 [X] NONE

Circumstances of Commitment Offense (Note any weapons used or possessed during the controlling or non-controlling offense):

According to Santa Clara County Probation Officer's Report, Santa Clara Police Department officers responded to a report of sexual assault which occurred at the Santa Clara University. The Victim (V1) stated she left her friend's dormitory at 2000 hours and proceeded to her dormitory. As she was walking, a man later identified as Brendan Newanforbi, approached her from behind, picked her up and carried her approximately five feet. Newanforbi dropped V1 on the ground and inserted his fingers into her vagina. V1 screamed and resisted Newanforbi so Newanforbi fled the scene. V1 was transported to the Valley Medical Center for a SART exam. V1 sustained abrasions to her left knee, left hip area, left wrist and complained of pain to her vagina. Officers reviewed surveillance footage from cameras and showed a figure running, and a grey ford mustang fleeing the scene. On 03/03/2017, Newanforbi was stopped for speeding and taken into custody for matching the description. Newanforbi self- admitted to groping V1 and stated he had uncontrollable urges and needed help.

Prior Criminal History: (Note any history of weapons, serious or violent felonies and any other relevant criminal information):

[X] Criminal History Attached (Dated no more than 30 days prior to due date of DRR)

01/12/18-PC 289(A)(1)(A): SEXUAL PENETRATION W/FORCE/ETC, PC 220(A)(1): ASSAULT:W/INTENT TO RAPE (6 YEARS PRISON)

Special Conditions of Parole: [X] CDCR Form 1515- Addendum Attached (* If necessary.)

[ ] None [ ] Anti-Narcotic Testing [ ] Drug Treatment [X] No Alcohol [ ] Attend POC [X] * Other

Parole Violations: (Note any parole violations, arrests, pending investigations and/or dispositions incurred during this period of review)

2025-03-04 FAILURE TO ATTEND SEX OFFENDER TREATMENT. COP (Verbal Reprimand)
2025-06-26 FAILURE TO ATTEND SEX OFFENDER TREATMENT. COP (Referral to PACT Program, Verbal Reprimand)
2025-07-15 FAILURE TO CHARGE GPS TWICE PER DAY EVERY 12 HOURS FOR 1 FULL HOUR OF CHARGING COP (Verbal Reprimand)
2025-08-18 FAILURE TO PROPERLY CHARGE GPS DEVICE COP (Referral to PACT Program, Verbal Reprimand)

Gang Activity Or Affiliation: (Note any past or present involvement in any prison, criminal, or street gang)

[X] None Validated: [ ] Member [ ] Associate [ ] Affiliate [ ] Dropout [ ] Active [ ] Inactive

Gang:

Current or past activity:

Mental Health Factors (Note mental health status of the parolee and his or her compliance with any mental health treatment):

[ ] None [ ] Voluntary [X] CCCMS [ ] EOP [ ] MDO

If CCCMS/EOP/MDO: [X] Compliant [ ] Non - Compliant

[X] BHR Report Attached (Dated no more than 45 days prior to due date of DRR)

Restitution (Note any fine and/or restitution balance at the time of review, and any effort to satisfy the fine and/or restitution balance):

[X] None [ ] Satisfied [ ] Not Satisfied Amount remitted to date: $ **0** Outstanding balance: $ **0**

Employment (Note any employment, educational or vocational training):

Employment [X] None [ ] Employed [ ] Full Time [ ] Part Time

Student: [X] None [ ] Student [ ] Full Time [ ] Part Time

DISCHARGE REVIEW REPORT

CDC Number:	Print Name (Print Last, First, MI):	Region:	Parole Unit:
BF3571	**NEWANFORBI, BRENDAN, NGEHSI**	**N**	**STOCKTON 2 GPS**

Employer Name (If applicable):

Position:

Program Name (If applicable):

DISCHARGE REVIEW REPORT

CDC Number: BF3571	Print Name (Print Last, First, MI): NEWANFORBI, BRENDAN, NGEHSI	Region: N	Parole Unit: STOCKTON 2 GPS

SECTION II PAROLEE DYNAMIC PROFILE CATEGORY: SA CSRA: 2

Objective 1: The parolee has maintained residence stability in a positive living environment.

1=Has been in the same pro-social living situation for eight months or longer; or less than eight months, but at least six months, when the most recent move was to improve overall living conditions, and will continue to be available to the parolee.

Objective 2: The parolee's time has been structured around pro-social activities geared towards self-reliance.

3=Time is unstructured and generally unaccounted for; involvement in pro-social activities is not evident.

Objective 3: The parolee has been compliant with his or her general and special conditions of parole.

3=The parolee has not met the rating standard for #1 or #2 above.

Objective 4: The parolee was referred and participated in programming for his or her top three criminogenic needs.

3=Failed to attend, participate, or complete.

Objective 5: The parolee has affiliated himself or herself with other pro-social individuals.

2=Parolee is seeking to terminate anti-social relationships, but still maintains some anti-social relationships.

Objective 6: Parolee has been referred to and participated in required sex offender treatment.

2=Attended and partially participated in sex offender treatment.

Parolee Dynamic Profile Recommendation: **Retain On Parole**

Aggravating Factor(s) to Consider:

Supervised Person Brendan Newanforbi is unemployed, incurred multiple parole violations during this review period, and has not completed sex offender treatment.

Mitigating Factor(s) to Consider:

Supervised Person Brendan Newanforbi has stable residence with his family.

STATE OF CALIFORNIA
DISCHARGE REVIEW REPORT
CDCR 1502-DR (Rev 01/22)

DEPARTMENT OF CORRECTIONS AND REHABILITATION
DIVISION OF ADULT PAROLE OPERATIONS

DISCHARGE REVIEW REPORT

CDC Number:	Print Name (Print Last, First, MI):	Region:	Parole Unit:
BF3571	NEWANFORBI, BRENDAN, NGEHSI	N	STOCKTON 2 GPS

SECTION III RECOMMENDATION

Parole Agent Recommendation:

- [ ] Retain on Parole
- [x] Discharge Denied/No Action
- [ ] Discharge Controlling Offense Approved
- [ ] Discharge Non-Controlling Offense Approved

Parole Agent Comments / Recommendation:

Objective 1: Residence - Newanforbi currently has stable residence with his family in Stockton.

Objective 2: Employment - Newanforbi is currently unemployed.

Objective 3: General/Special Conditions - Newanforbi has incurred multiple parole violations during this review period.

Objective 4: Addressed Criminogenic Needs - Newanforbi's Criminogenic Needs for Substance Abuse rated Probable and Unlikely for all other categories.

Objective 5: Pro-Social - Newanforbi does not appear to have a large social circle.

Objective 6: Sex Offender - Newanforbi is not currently titrated and is not in the maintenance phase of his treatment with HOPE.

Supervised Person Brendan Newanforbi is a 29-year-old first termer committed to the California Department of Corrections and Rehabilitation for Penal Code 289(a)(1)(A) Penetration w/Force/Violence/Fear of Immediate Bodily Injury for 6 years in state prison. Newanforbi appears to have stable residence with his family in Stockton, CA. Newanforbi is currently unemployed and not attending school. Newanforbi has incurred multiple parole violations during this review period for failure to attend sex offender treatment and failure to charge Global Positioning System. Newanforbi is currently attending sex offender treatment and has not completed treatment. Based on Sex Offender Management Program Earned Discharge Criteria, Policy 19-03 (Amended), Newanforbi is not eligible for EDC at this time due to having a 10-year parole term. Newanforbi has only completed approximately 4 years of the required 6.5 years of supervision. Continued supervision is both warranted and recommended at this time. Recommend discharge be denied at this time.

Parole Agent (Print Name):	Electronically Signed by:	Badge #:	Date Signed:
LONG MOUA	LONG MOUA	7472	11-14-2025

Unit Supervisor Actions:

- [x] Review
- [x] Decision
- [ ] Retain On Parole
- [x] Discharge Denied/No Action
- [ ] Refer to BPH
- [ ] Discharge Controlling Offense Approved
- [ ] Discharge Non-Controlling Offense Approved
- [x] Refer to Parole Administrator

Unit Supervisor Comments/ /Decision /Recommendation:

The subject should be commended for maintaining compliance with parole supervision. The subject is unemployed, and the subject lives with his family in Stockton. The subject has no restitution obligations and, the subject has had five parole violations this period. Subject's Sex Offender Treatment Progress (HOPE): 10/15/2024 Stable 6/LSCMI 5. Level 2 below average Risk. Missed 6 sessions this Parole Period. Core Phase of treatment and working in Block 1 Module 4/7. Subject has presented homework on Victim Empathy. Does take full responsibility of offense. Subject has a relapse prevention plan to assist in his alcohol and marijuana use. Current focus on problem solving and relationship stability focusing on healthy relationships. Not eligible for titration at this time. Currently scheduled for 2 groups a week and 1 monthly individual. HOPE SOT recommends continued treatment. Based on Sex Offender Management Program Earned Discharge Criteria, Policy 19-03 (Amended), Subject is not eligible for EDC at this time due to having a 10-year parole term. Eagan has only completed approximately 4 years of the required 6.5 years of supervision. I concur with the Parolee Dynamic Profile and AOR's recommendation "Retain on Parole". Continued DAPO supervision is warranted and recommended.

Unit Supervisor (Printed Name):	Electronically Signed by:	Badge #:	Date Signed:
GARY NOGUCHI	GARY NOGUCHI	7284	11-19-2025

Parole Administrator Actions:

- [x] Review
- [x] Decision
- [ ] Retain On Parole
- [x] Discharge Denied/No Action
- [ ] Refer To BPH
- [ ] Discharge Controlling Offense Approved
- [ ] Discharge Non-Controlling Offense Approved

Parole Administrator Comments / Decision/ Recommendation:

Concur with the Unit Supervisor, continued parole supervision is warranted and recommended at this time.

DISCHARGE REVIEW REPORT

CDC Number:	Print Name (Print Last, First, MI):	Region:	Parole Unit:
BF3571	**NEWANFORBI, BRENDAN, NGEHSI**	**N**	**STOCKTON 2 GPS**

Parole Administrator (Print Name):	Electronically Signed by:	Badge #:	Date Signed:
KEELY DODD	**KEELY DODD**	**6462**	**11-19-2025**

Parolee Provided Copy (Date)

☐ Mailed By (Initials):

☐ Delivered By (Initials):

AGENT OF RECORD CONTACT INFORMATION AS OF 11/19/2025

AGENT NAME: ROJO, JOSEELYN

AGENT CONTACT: JOSEELYN.ROJO@CDCR.CA.GOV, (510) 871-7372 (Cell)

PAROLE UNIT: STOCKTON 2 GPS, 612 CARLTON AVENUE, STOCKTON, CA, 95203, (209) 948-7652

EXHIBIT E

CALIFORNIA BOARD OF BEHAVIORAL SCIENCES
COMPLAINT AGAINST LICENSED MARRIAGE AND FAMILY THERAPIST

Licensee: KATHERINE A. PALACIOS
License Type: Licensed Marriage and Family Therapist (LMFT)
License Number: 44555
License Status: License Renewed & Current (Exp. February 28, 2027)
Board: California Board of Behavioral Sciences
Incident Date: In/On or about August 2025
Location: Stockton, San Joaquin County, California

Complainant: Brendan Ngehsi Newanforbi
Address: 1455 Cat Tail Drive, Stockton, CA 95204
Phone: (209) 801-4442
Email: bnn.cdl@gmail.com

COMPLAINT DESCRIPTION

I. INTRODUCTION

I am filing this complaint against Katherine A. Palacios, LMFT, License Number 44555, who serves as the Clinical Director of the HOPE Program (operated by HOPE Psychotherapy, Inc.) located at 5403 N. Pershing Avenue, Suite 200, Stockton, CA 95207. HOPE is a CASOMB-certified sex offender treatment provider that contracts with the California Department of Corrections and Rehabilitation ("CDCR") to provide mandatory treatment services under California Penal Code § 3008(d). As a licensed therapist exercising delegated governmental authority over liberty-affecting treatment determinations, Ms. Palacios is subject to the professional and ethical standards governing LMFTs under the California Business and Professions Code and regulations administered by the Board of Behavioral Sciences.

This complaint arises from two categories of professional misconduct: (1) false, defamatory, and retaliatory statements Ms. Palacios made about me during a Case Conference/Containment Team Meeting ("CTM") held in/on or about August 2025 at a CDCR parole office in Stockton, California; and (2) Ms. Palacios's ongoing pattern of applying a blanket retention policy—rather than individualized clinical assessments—to determine my treatment status, in violation of her professional duties as a licensed therapist.

II. THE AUGUST 2025 CASE CONFERENCE INCIDENT — DEFAMATORY AND RETALIATORY STATEMENTS

A. Setting and Attendees. In/on or about August 2025, I was directed to report to the CDCR parole office in Stockton, California, for a Case Conference/Containment Team Meeting. Upon

arrival, I was subjected to a pat-down search by parole agents before being escorted into the meeting room. The following individuals were present:

(a) Agent Chow, Parole Agent, CDCR;
(b) Agent G. Noguchi #7284, Supervising Parole Agent, CDCR (incoming supervisor);
(c) Agent A. Derrick #6910, Parole Agent, CDCR (my assigned parole agent);
(d) The then-current supervising parole agent (now retired, name unknown); and
(e) Defendant Katie A. Palacios, LMFT, Director, HOPE Program.

B. Ms. Palacios's Defamatory Statements. Prior to Ms. Palacios's remarks, I conducted myself in a respectful and cordial manner throughout the meeting. Ms. Palacios herself acknowledged this fact at the outset of her remarks. When it came time for Ms. Palacios to address the meeting, she directed her remarks at me and stated, in substance:

> *"Mr. Newanforbi, I am impressed with your performance today. You present a completely opposite side at the HOPE group, wouldn't you agree?"*

I responded by stating: "I 100% disagree with that statement."

Ms. Palacios then continued, making the following statements in the presence of all CTM attendees:

> *"You're disruptive during HOPE group sessions, often deviating from the curriculum to talk how you disagree with the treatment. You're a ringleader, because when you start complaining then the other guys chime in. Your behavior is abhorrent and you're often disrespectful to my staff. Quite frankly I'm shocked at the acting job you've put on today, it is quite frankly impressive."*

C. The Statements Were False. Ms. Palacios's statements were false in the following material respects:

"You're disruptive." I was not disruptive. I raised legitimate concerns about program conditions during HOPE group sessions in 2024 and 2025, including my prolonged retention in the program despite completion of all requirements, the repeated loss of my completed homework by program staff, and the disruption of my progress caused by repeated clinician turnover and resulting group resets. These complaints constituted constitutionally protected speech addressing matters of public concern. Ms. Palacios falsely recharacterized protected expression as clinical misconduct.

"You're a ringleader." I was not a ringleader. I expressed my own views during group sessions. Other participants who independently agreed with my concerns chose to voice their own opinions. Ms. Palacios falsely attributed the independent speech of other participants to my influence, creating the false impression that I was orchestrating group dissent or insubordination. Notably, Ms. Palacios herself admitted the basis: "because when you start complaining then the other guys chime in"—i.e., others voluntarily agreed.

"Your behavior is abhorrent." My behavior was not abhorrent. I had maintained compliance with my parole conditions for approximately four and a half years, completed all assigned HOPE Program coursework multiple times, and had no major parole violations. The word "abhorrent"—meaning extremely bad, repugnant, or detestable—was a gross and false characterization of my conduct.

"You're often disrespectful to my staff." I was not disrespectful to HOPE Program staff. I raised legitimate concerns in a manner appropriate to the group setting. Ms. Palacios conflated disagreement and advocacy with disrespect.

"I'm shocked at the acting job you've put on today." I was not acting. My respectful and cordial demeanor at the CTM meeting was genuine and consistent with my character as observed by other parole agents, including Agent Chow, who had interacted with me extensively and knew me to be respectful and professional. By characterizing my genuine conduct as a performance, Ms. Palacios communicated to all officials present that I was fundamentally deceptive and that my good behavior could not be trusted—a false statement about my character designed to undermine my credibility with the very officials who controlled my liberty.

D. Agent Chow's Reaction Confirms the Falsity. Immediately after Ms. Palacios concluded her statements, Agent Chow—a CDCR parole agent who had interacted extensively with me over a sustained period—looked at me and stated, in substance: "Newanforbi, what's going on?" Agent Chow's visible surprise at Ms. Palacios's characterization demonstrated that her description of me as "abhorrent," "disruptive," and a "ringleader" did not match the person Agent Chow knew through extensive firsthand experience.

E. Ms. Palacios Acted With Malice and Retaliatory Intent. Ms. Palacios's statements were made with actual malice—knowledge of their falsity or reckless disregard for the truth. She acknowledged my respectful conduct at the CTM meeting and then deliberately reframed it as deception. The "ringleader" label was explicitly tied to my protected complaints. Following the CTM meeting and the resulting escalation of my parole supervision, I curtailed my speech during HOPE group sessions out of fear of further retaliation. Ms. Palacios subsequently remarked during group sessions that I was "doing better" and stated she had been "putting in a good word" for me with my parole agent—confirming that she viewed my exercise of free speech as the problem and my silence as the solution. This demonstrates her characterizations were not good-faith clinical assessments but were designed to punish and suppress my protected expression.

III. IMMEDIATE AND CONCRETE HARM FROM THE AUGUST 2025 STATEMENTS

Escalation of parole supervision. Immediately following Ms. Palacios's statements, Agent Derrick recommended that my supervision level be increased from once per month to twice per month. The supervising parole agent present at the meeting approved this escalation on the spot. This doubling of my supervision frequency was a direct result of Ms. Palacios's characterization of me as "abhorrent," "disruptive," and a "ringleader."

Incorporation into official CDCR reports. Ms. Palacios's characterizations of me as "disruptive" and a "ringleader" were subsequently incorporated into the CDCR Form 1502-DR Discharge Review Report prepared on or about November 12, 2025. This report relied on input from both Ms. Palacios and Agent Derrick and recommended my continued retention on parole, directly extending state supervision over my liberty based in substantial part on Ms. Palacios's false characterizations.

Destruction of credibility with parole officials. By telling the room that my respectful conduct was an "acting job," Ms. Palacios communicated to every official present—including my current parole agent, his incoming supervisor, and the presiding supervisor—that I was fundamentally deceptive, making it impossible for me to be believed in future interactions regardless of my actual conduct.

Chilling of protected speech. Following the CTM meeting, I curtailed my speech during HOPE group sessions, abandoned written complaints, and experienced physiological symptoms of anxiety including increased heart rate, shaking hands, and nervousness during parole interactions.

Emotional distress. Ms. Palacios's statements caused me significant emotional distress, including anxiety, humiliation, depression, sleep disruption, hypervigilance, and fear of speaking. I was effectively shut down during the CTM meeting with no meaningful opportunity to respond to her attack on my character in front of the officials who controlled my liberty.

IV. APPLICATION OF A BLANKET RETENTION POLICY IN LIEU OF INDIVIDUALIZED CLINICAL ASSESSMENT

A. The Blanket Retention Policy. Beyond the August 2025 incident, Ms. Palacios's professional misconduct extends to her ongoing implementation of HOPE's blanket retention policy—an official policy of refusing to certify "completion" for any parolee prior to discharge from parole, regardless of clinical progress, compliance, risk reduction, or time in treatment. This policy was documented under oath by HOPE's own Executive Director of Clinical Training in the case of *In re Frederick Dwyer*, No. SC-080355A (San Mateo Superior Court, March 20, 2023), where the court found this practice "vitiates the Legislature's directive" under Penal Code § 3008(d)(2). Despite this judicial condemnation, Ms. Palacios has continued applying the identical policy to my treatment through November 2025 and beyond.

B. Misrepresentation of Policy-Driven Predeterminations as Individualized Clinical Assessments. Ms. Palacios represented to CDCR—through treatment status reports, CTM inputs, and the November 2025 Discharge Review Report—that I have "not completed" sex offender treatment. This characterization was not the product of an individualized clinical assessment of my progress, risk reduction, or compliance. It was the automatic output of HOPE's blanket retention policy, which makes completion impossible for all participants prior to parole discharge. Ms. Palacios knew this characterization did not reflect my individual clinical performance because the policy forecloses completion for every participant without exception. As a licensed therapist, Ms. Palacios has a professional and ethical duty to provide individualized clinical assessments—not to misrepresent policy-driven predeterminations as clinical judgments to governmental agencies that rely on those representations to make liberty-affecting decisions.

C. Violation of Regulatory Requirements. Effective April 17, 2025, Title 15 CCR § 3574(f)(1) explicitly provides that "Treatment completion and parole discharge are legally separate decisions." A treatment provider that refuses to certify completion until discharge from parole collapses these separate decisions into one, in direct violation of the regulation. Despite this regulatory mandate—enacted specifically to address the deficiencies identified in the *Dwyer* ruling—Ms. Palacios has continued to apply HOPE's blanket retention policy through November 2025 and beyond.

V. OPERATION OF AN ARBITRARY TREATMENT STRUCTURE WITH NO PUBLISHED CRITERIA

Under Ms. Palacios's direction as Clinical Director, HOPE's treatment structure requires mandatory phase repetitions with no basis in statute or regulation. Participants must complete the "Core" phase twice, the "Maintenance" phase twice, and then a "titration" phase before being

considered for program completion. This framework appears nowhere in Penal Code § 3008(d) or Title 15 CCR § 3574, and results in participants remaining in treatment for 5–7+ years with no meaningful prospect of completion.

Ms. Palacios, as Clinical Director, has never provided me with:

(f) Written criteria for program completion;
(g) A written explanation of what conduct or progress is required to advance through phases;
(h) An appeal process for titration or phase-advancement decisions;
(i) Any mechanism to challenge lost homework or forced phase restarts.

Ms. Palacios has admitted that homework assignments are "routinely lost" within the HOPE Program, yet participants—including me—are forced to restart phases due to staff turnover or job-related group changes, forfeiting progress previously earned. The combined effect is that my characterization as "non-complete" was predetermined from the day I entered the program and bears no relationship to my individual clinical progress, compliance, or risk level.

VI. FINANCIAL CONFLICT OF INTEREST UNDERMINING CLINICAL OBJECTIVITY

HOPE operates under a fee-for-service billing model. Per the Alameda County Probation Department Procurement Contract No. 26374 (First Amendment approved January 9, 2024), HOPE bills: $100.00 per group psychotherapy session (90 minutes); $100.00 per Collaborative Model Team meeting (per client staffed, 90 minutes); $200.00 per individual psychotherapy session (60 minutes); $500.00 per Polygraph Examination; $1,800.00 per psychosexual evaluation; and $125.00 per quarterly treatment plan.

A single parolee attending two group sessions per week generates approximately $10,600 annually in baseline group and CTM fees alone. Over my 10-year parole term, this represents approximately $106,000 in baseline revenue from my continued enrollment. HOPE's contracts provide no documented compensation, bonus, or incentive for successful treatment completion. Revenue terminates upon discharge. Ms. Palacios, as Clinical Director, controls the "completion" determination that CDCR uses as a gatekeeper for discharge eligibility, while possessing a direct, ongoing financial interest in ensuring that determination is never favorable. This conflict of interest compromises the clinical objectivity required of a licensed marriage and family therapist and raises serious ethical concerns under the professional standards governing LMFTs.

VII. PROFESSIONAL AND ETHICAL STANDARDS VIOLATED

Ms. Palacios's conduct raises serious concerns under the professional and ethical standards governing Licensed Marriage and Family Therapists, including but not limited to:

1. **Duty to provide individualized clinical assessments:** Ms. Palacios applied a blanket retention policy to determine my treatment status rather than conducting an individualized assessment of my clinical progress, compliance, risk reduction, or treatment needs. A licensed therapist exercising delegated authority over a client's liberty

has a heightened duty to ensure that clinical determinations are individualized, evidence-based, and not the product of administrative convenience or financial incentive.

2. **Duty to maintain professional objectivity free from financial conflicts:** Ms. Palacios controls the treatment completion determination while operating under a billing model that financially rewards retention and penalizes discharge. This undisclosed conflict of interest compromises the objectivity required of a licensed therapist and creates a structural incentive to provide unfavorable treatment characterizations.
3. **Duty not to misrepresent clinical determinations:** Ms. Palacios represented to CDCR that my "non-completion" characterization was the product of clinical assessment when she knew it was the automatic output of a blanket policy applied uniformly to every participant. Misrepresenting policy-driven predeterminations as individualized clinical judgments to governmental agencies constitutes a form of professional dishonesty.
4. **Duty not to retaliate against clients for protected expression:** Ms. Palacios made false and defamatory statements about me to parole officials in direct response to my constitutionally protected complaints about program conditions. She then confirmed the retaliatory nature of her conduct by praising me when I ceased exercising those rights. Using the therapist-client relationship to punish protected speech and suppress advocacy constitutes an abuse of professional authority.
5. **Duty to provide transparent treatment criteria:** Ms. Palacios operated a treatment program with unpublished, internally contradictory criteria for phase progression, titration, and completion. She failed to provide me with written criteria, a meaningful path to completion, or any appeal or challenge mechanism. A licensed therapist has a duty to provide clients with transparent, achievable treatment goals.
6. **Duty to maintain accurate records:** Ms. Palacios admitted that homework assignments are "routinely lost" within the HOPE Program, yet she failed to implement any safeguards against record loss and forced participants to restart treatment phases due to her program's administrative failures. A licensed therapist has a duty to maintain accurate and complete records of client progress.
7. **Duty to comply with applicable law and regulation:** Ms. Palacios continued to apply HOPE's blanket retention policy after: (a) the San Mateo Superior Court's March 2023 order in *In re Frederick Dwyer* specifically condemned the policy; and (b) the April 2025 amendments to Title 15 CCR § 3574(f)(1) enacted to address the deficiencies. A licensed therapist has a duty to conform her practice to applicable law and regulatory requirements.

VIII. SPECIFIC HARM CAUSED BY MS. PALACIOS'S CONDUCT

As a direct result of Ms. Palacios's professional conduct, I continue to be subjected to custodial parole supervision conditions including: continuous GPS monitoring; curfew restrictions; geographic movement limitations; mandatory HOPE treatment attendance (multiple sessions per week); reporting requirements to parole officers; restrictions on employment, travel, and association; and the requirement to maintain CDCR-approved housing.

These conditions have specifically and directly prevented me from pursuing full, unrestricted employment as a Commercial Driver's License (CDL) holder. My documented lost wages total

no less than $109,408 for the years 2022–2024, representing the difference between my but-for CDL earning capacity of $72,000 per year and my actual supervision-suppressed W-2 earnings of $39,553 (2022), $11,145 (2023), and $55,894 (2024). These losses continue to accrue at approximately $6,000 per month ($72,000 annually) for each month I remain unable to pursue unrestricted CDL employment due to Ms. Palacios's conduct.

I have also suffered severe emotional distress, including: pervasive anxiety from continuous GPS monitoring imposed based on Ms. Palacios's predetermined treatment characterization; humiliation and stigma from being classified as having "not completed" treatment under a system where completion is unattainable; chronic sleep disturbance and hypervigilance arising from the cumulative stress of arbitrary custodial control; psychological harm from the knowledge that my continued custody rests on a misrepresentation; and ongoing distress from entrapment in a treatment system that profits from my continued enrollment with no achievable exit.

IX. SUPPORTING DOCUMENTATION AVAILABLE

I can provide the following documentation upon request:

8. The Order in *In re Frederick Dwyer*, No. SC-080355A (San Mateo Superior Court, March 20, 2023), documenting HOPE's blanket retention policy through sworn testimony of HOPE's own Executive Director of Clinical Training;
9. Relevant excerpts of the Alameda County Probation Department Procurement Contract No. 26374 (First Amendment), showing HOPE's billing rates and payment terms;
10. CDCR Form 1502-DR Discharge Review Report dated November 12, 2025, incorporating HOPE's treatment characterization and the resulting "Retain on Parole" recommendation;
11. My filed Complaint for Damages in San Joaquin County Superior Court, Case No. STK-CV-UF-2026-1032 (Newanforbi v. HOPE Psychotherapy, Inc., et al.), asserting claims for fraud, negligence, false imprisonment, intentional infliction of emotional distress, and interference with prospective economic advantage;
12. My filed Complaint for Damages (Defamation — Slander Per Se and Libel Per Se) against Katie A. Palacios in San Joaquin County Superior Court, arising from the August 2025 CTM statements;
13. W-2 wage statements documenting supervision-suppressed CDL earnings for 2022–2024;
14. BBS License Verification showing Ms. Palacios's active LMFT License No. 44555; and
15. My First Amended Complaint and service documentation in Newanforbi v. Derrick, et al., Case No. 2:26-cv-00004-DJC-JDP (E.D. Cal.), which includes claims arising from the same course of conduct.

X. REQUEST FOR INVESTIGATION

I respectfully request that the Board of Behavioral Sciences investigate Ms. Palacios's conduct and take appropriate disciplinary action. Ms. Palacios is a licensed therapist who exercises delegated governmental authority over the liberty of individuals under state supervision. Her conduct—applying a blanket retention policy rather than individualized clinical assessments, misrepresenting policy-driven predeterminations as clinical judgments, retaliating against a client for protected speech, operating a treatment program with no published criteria or appeal mechanisms, and maintaining an undisclosed financial conflict of interest that incentivizes

indefinite retention—represents a serious departure from the professional and ethical standards required of a Licensed Marriage and Family Therapist.

The harm caused by Ms. Palacios's conduct is ongoing. Every day that HOPE continues to apply its blanket retention policy to my treatment—and every treatment characterization Ms. Palacios submits to CDCR based on that policy—constitutes a new act of professional misconduct that perpetuates my continued custodial supervision, economic harm, and emotional distress.

Respectfully submitted,

/s/ Brendan Ngehsi Newanforbi
BRENDAN NGEHSI NEWANFORBI
1455 Cat Tail Drive
Stockton, CA 95204
(209) 801-4442
bnn.cdl@gmail.com
Date: February 21, 2026